

Patient Reference ID: P-00038

Diagnostic Time: 2026-07-07 08:00 UTC

Analysis Engine: ClinicalInference-LR-v2.1

Verification Signature: NH-38-41496

I. Cardiovascular Risk Assessment

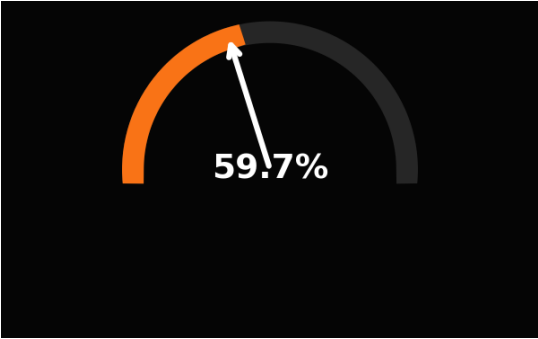
Cardiovascular Evaluation

Risk Category: **HIGH RISK (59.7%)**

Clinical Severity: **Moderate**

Key Patient Metrics:

- Age: 55.0 years
- Biological Sex: Male
- Chest Pain Type: Typical Angina
- Resting Blood Pressure: 140.0 mmHg
- Serum Cholesterol: 250.0 mg/dL
- Fasting Blood Sugar: Normal (≤ 120 mg/dL)
- Resting ECG Result: ST-T Wave Abnormality
- Max Heart Rate Achieved: 150.0 bpm



II. Clinical Consultation Notes & Signature

Consulting Physician Remarks:

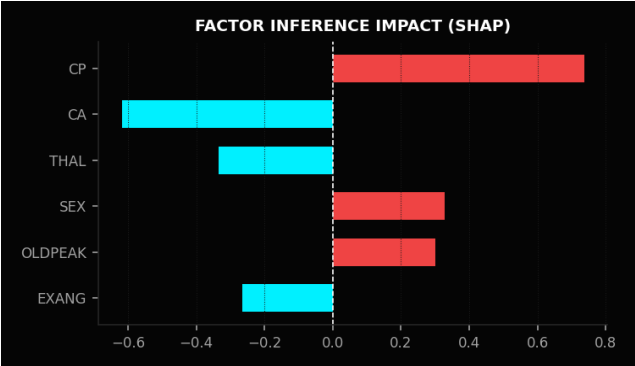


Authorized Signature: _____ Date: _____

III. AI Pathology Diagnostics (SHAP Factors)

Patient Summary: Diagnostics reveal a high risk profile (59.7% confidence). The profile is primary driven by chest pain type, biological sex. Seeking physician guidance is recommended.

Medical Assessment: Myocardial ischemia markers are positive. Significant arterial vessel calcification is suspected.



IV. Recommended Treatment Pathway & Pathway Timeline

Recommended Diagnostic Tests	Referrals & Specialists	Lifestyle & Dietary Adjustments
- Standard 12-lead Electrocardiogram (ECG) - Complete Fasting Lipid Panel - Stress Echocardiography - Coronary Computed Tomography Angiography (CCTA) - Lipoprotein(a) and ApoB screening.	- Primary Care Physician (PCP) - Board-certified Cardiologist - Registered Dietitian (RD)	- Maintain a low-sodium, high-fiber cardiovascular diet (DASH diet framework). - Engage in moderate-intensity aerobic exercise (e.g., brisk walking 150 minutes/week). - Complete cessation of any nicotine consumption. - Reduce alcohol intake to fewer than 1 standard drink daily.

Recommended Follow-up Timeline: Within 48 to 72 hours

AI CLINICAL DISCLAIMER: NEUROHEART AI is a clinical decision support system. Calculations are probabilistic and should not replace professional medical diagnosis, advice, or therapy.